

Steroid-sparing treatment: safety and escalation

Part 2 | Escalation begins with a safety diagnosis

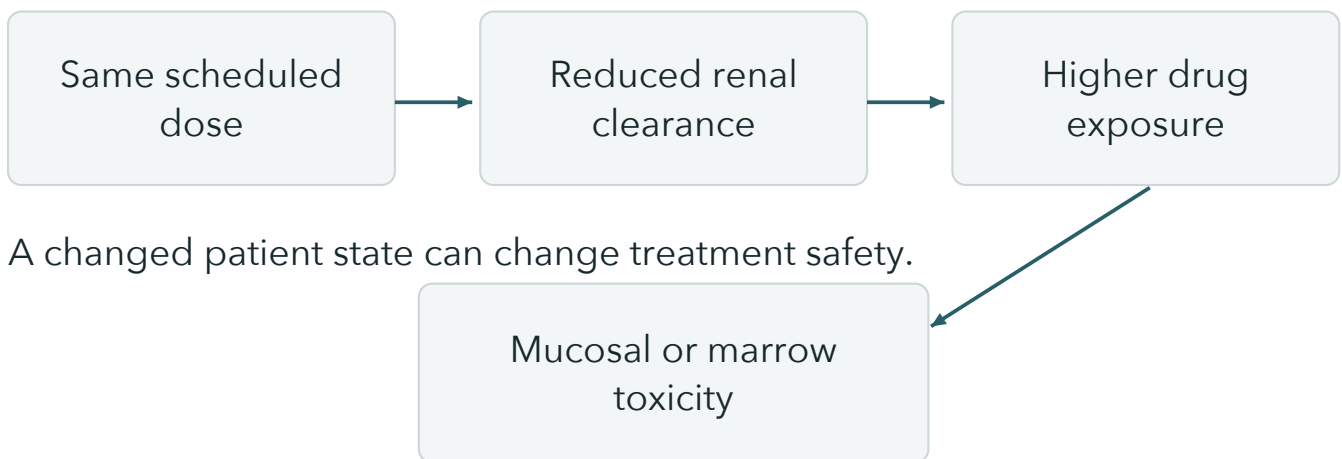
Identify what continuing steroid is costing and what it is treating.

Question	Decision relevance
Is infection adequately excluded?	Immunosuppression may worsen its course
Is the phenotype or anatomy discordant?	Revisit masquerade or mechanical disease
Is inflammation controlled only with harmful steroid?	Consider a steroid-sparing strategy
Can monitoring and adherence be sustained?	Choice must be feasible and safe

Choose therapy jointly with relevant systemic specialists. Define an ocular endpoint and a toxicity endpoint before starting. A drug that improves cells while causing progressive pressure damage is not an uncomplicated success. Treatment failure can reflect inadequate exposure, incorrect diagnosis or irreversible structural damage, rather than insufficient immunosuppression alone.

Methotrexate toxicity can follow a change elsewhere

Correct weekly dosing does not prevent toxicity when clearance falls.



Original mechanism map: unchanged dose can produce higher exposure when elimination falls.

Review renal function, blood counts, liver tests and interacting drugs according to the treatment protocol. Mucositis and cytopenia require urgent assessment. A new renal insult can reduce methotrexate elimination even without an accidental dosing increase. Confirm the weekly schedule explicitly; monitoring is a continuing process rather than a one-time pre-treatment screen.

Azathioprine has metabolic and interaction constraints

An enzyme pathway can change exposure without changing the prescription.

Constraint	Reasoning consequence
Thiopurine metabolic susceptibility	Consider appropriate pretreatment assessment
Allopurinol co-prescription	Raises risk through reduced inactivation
Marrow suppression or infection	Requires prompt clinical and laboratory review
Normal baseline blood count	Does not remove the need for follow-up

Azathioprine requires specialist adjustment when an interacting xanthine-oxidase inhibitor is needed; simply adding the second drug at usual doses can be dangerous. Prescribing-information precautions and local monitoring protocols take precedence over a generic uveitis regimen. Pharmacogenetic assessment does not replace blood-count monitoring.

Anti-TNF safety depends on the patient phenotype

A useful class can be unsuitable in a particular systemic context.

Context	Safety question
Possible demyelinating disease	Seek relevant assessment before selection
Latent or active infection risk	Screen and manage before immunosuppression
New systemic infection during treatment	Reassess treatment and infection urgently
Apparent biologic nonresponse	Revisit diagnosis, exposure and true activity

TNF has a role in host containment of tuberculosis; neutralization changes infection risk. Animal granuloma experiments explain a mechanism, not a universal human histological outcome. Drug-label screening and adverse-effect warnings should guide care. An effective uveitis drug is not automatically the safest drug for every neurological or infectious history.

Follow the eye and the treatment together

Separate recurrent activity from drug toxicity and residual damage.

Apparent problem	Reassessment target
Blur with quiet anterior chamber	Macula, cataract and optical causes
Raised IOP	Inflammation, angle change and steroid response
Persistent cells after escalation	Diagnosis, adherence and exposure
New laboratory abnormality	Drug toxicity and competing illness

Document the intended follow-up interval and what will trigger urgent contact. Structural imaging can show a cause of reduced vision that will not respond to further systemic dosing. Tapering, switching and withdrawal should be planned around disease control and systemic safety, rather than using a single good visit as proof of durable remission.